

AbbVie Interview Master Prep

Director, Government Affairs - U.S. Reimbursement

Prepared for Joseph Stewart on April 10, 2026.

1. Executive Summary

AbbVie's Medicare exposure is now a combined Part B, Part D, and cross-benefit strategy problem. As of April 10, 2026, Imbruvica is under negotiated pricing for IPAY 2026, Vraylar and Linzess are under negotiated pricing for IPAY 2027, and Botox / Botox Cosmetic has been selected for IPAY 2028 across Parts B and D. That means AbbVie is no longer managing negotiation as a future issue. It is already managing one completed cycle, one active implementation cycle, and one upcoming Part B-sensitive cycle at the same time. AbbVie 2025 Form 10-K (<https://www.sec.gov/Archives/edgar/data/1551152/000155115226000008/abbv-20251231.htm>), CMS IPAY 2026 negotiated prices (<https://www.cms.gov/files/document/fact-sheet-negotiated-prices-initial-price-applicability-year-2026.pdf>), CMS IPAY 2027 negotiated prices (<https://www.cms.gov/files/document/fact-sheet-negotiated-prices-ipay-2027.pdf>), CMS IPAY 2028 selected drugs (<https://www.cms.gov/files/document/factsheet-medicare-negotiation-selected-drug-list-ipay-2028.pdf>)

For interview purposes, the most important AbbVie products to know are Botox, Skyrizi, Elahere, Imbruvica, Vraylar, Linzess, Rinvoq, Venclexta, Duopa, and Ozurdex. Botox matters because it is the clearest live Part B reimbursement issue. Skyrizi matters because it crosses from IV induction to subcutaneous maintenance. Elahere matters because it is a classic oncology buy-and-bill product with visible HCPCS and ASP exposure. Duopa matters because it is one of the strongest portfolio examples for hospital, procedural, and DME-adjacent reimbursement complexity. AbbVie 2025 Form 10-K (<https://www.sec.gov/Archives/edgar/data/1551152/000155115226000008/abbv-20251231.htm>), CMS April 2026 Part B payment limits (<https://www.cms.gov/files/zip/april-2026-medicare-part-b-payment-limit-files-03-30-2026-final-file.zip>)

The role itself is not a generic federal policy seat. The posting reads like a portfolio-level reimbursement strategy role that sits between CMS engagement, product reimbursement execution, and commercial translation. The person in the job has to understand coding, coverage, ASP, discarded-drug refunds, physician-administered launch planning, provider-society engagement, and how broader federal policy changes create product-level access consequences. AbbVie role notes (.././abbvie)

Joseph Stewart's strongest angle is not claiming direct AbbVie ownership. It is showing that he already operates in the same operating logic: policy to reimbursement mechanics to provider behavior to access and commercial consequence. The bridge is strongest on Medicare fluency, reimbursement strategy, payer and formulary analytics, launch readiness, and provider economics. The bridge is weaker on direct AbbVie-like buy-and-bill ownership, and that should be addressed directly and credibly. [app/about/page.tsx](#) (.././app/about/page.tsx), [portfolio_brief.md](#) (.././portfolio_brief.md), [Executive Summary.pdf](#) (/Users/josephstewart/Documents/Executive%20Summary.pdf)

2. What This Role Actually Owns

Role mandate

- Monitor Medicare reimbursement policy developments with a product-specific lens.
- Build comprehensive product coverage and reimbursement strategies for Medicare.
- Identify Medicare Part B threats and opportunities across AbbVie's portfolio.
- Represent AbbVie before CMS and other federal agencies.
- Partner across Government Affairs to execute advocacy strategies aligned with enterprise goals and product reimbursement needs.
- Support launch planning, label-change planning, and commercial readiness with reimbursement guidance.

What to signal in the interview

- Product-specific Medicare strategy, not abstract policy commentary.
- CMS engagement fluency tied to coding, coverage, ASP, payment, and implementation milestones.
- Comfort operating across Part B, Part D, and mixed-benefit products.
- Ability to translate federal policy into provider behavior, payer behavior, and access execution.
- Cross-functional leadership with legal, market access, finance, medical, brand, and government affairs teams.

What the job likely produces

- Medicare product heat maps.
- CMS comment letters and negotiation-position materials.
- Coding and reimbursement playbooks for launches and label changes.

- ASP and payment-limit tracking models.
- Provider-society issue briefs.
- Portfolio-wide risk memos on discarded-drug policy, negotiation, and site-of-care shifts.

3. AbbVie Portfolio: What Matters Most

Product	Benefit orientation	Why it matters
Botox	Part B dominant; selected for Parts B and D negotiation	Highest-value current Part B reimbursement issue; coding, waste, ASP, and negotiation all matter.
Skyrizi	Mixed / cross-benefit	IV induction plus SC maintenance makes it the cleanest cross-benefit AbbVie product.
Elahere	Part B	Provider-administered oncology product with visible J-code and ASP exposure.
Imbruvica	Part D	AbbVie's completed IRA implementation example.
Vraylar	Part D	Major neuroscience brand already selected for IPAY 2027.
Linzees	Part D	Major GI brand already selected for IPAY 2027.
Rinvoq	Part D	High-value growth brand and plausible future negotiation risk.
Venclexta	Part D with regimen-level clinic relevance	Important because reimbursement strategy has to reflect oncology regimen economics, not just tablet benefit classification.
Duopa	Hospital / procedural / DME-adjacent complexity	Strongest portfolio example for why Part A and hospital reimbursement knowledge still matters.
Ozurdex	Part B / facility-administered	Useful example of facility and procedural reimbursement complexity outside classic infusion oncology.

Current public Part B payment evidence

Product	HCPCS	April 1, 2026 to June 30, 2026 payment limit
Botox	J0585	\$6.512 per unit
Skyrizi IV	J2327	\$14.599 per mg
Elahere	J9063	\$71.128 per mg

Source: CMS April 2026 Part B payment limits

(<https://www.cms.gov/files/zip/april-2026-medicare-part-b-payment-limit-files-03-30-2026-final-file.zip>)

4. IRA and Part B Negotiation: AbbVie-Specific Readout

Product	Status	What to say
Imbruvica	IPAY 2026 negotiated	AbbVie already has live negotiation implementation experience.
Vraylar	IPAY 2027 negotiated	AbbVie has a second-cycle neuroscience brand already in scope.
Linzees	IPAY 2027 negotiated	AbbVie has a second-cycle GI brand already in scope.
Botox / Botox Cosmetic	IPAY 2028 selected	Botox is the major Part B-sensitive negotiation planning issue now.

Why Botox matters operationally

- CMS selected Botox / Botox Cosmetic on January 27, 2026 for IPAY 2028. CMS IPAY 2028 selected drugs (<https://www.cms.gov/files/document/factsheet-medicare-negotiation-selected-drug-list-ipay-2028.pdf>)
- Botox is provider-administered, billed by HCPCS unit, supplied in single-dose vials, and tied to discarded-drug workflow. FDA BOTOX label (https://www.accessdata.fda.gov/drugsatfda_docs/label/2023/103000s53271bl.pdf), CMS discarded-drug article (<https://www.cms.gov/medicare-coverage-database/view/article.aspx?articleId=56646>)

- That means negotiation can affect not just federal pricing but payment-limit logic, provider reimbursement, operational billing, and AbbVie's discarded-drug refund exposure. CMS IPAY 2028 final guidance (<https://www.cms.gov/files/document/ipay-2028-final-guidance.pdf>), CMS CY 2026 PFS fact sheet (<https://www.cms.gov/newsroom/fact-sheets/calendar-year-cy-2026-medicare-physician-fee-schedule-final-rule-cms-1832-f>)

5. Botox Coding, Waste, and Payment: Interview Deep Dive

Core coding structure

For therapeutic Botox, the drug line is HCPCS J0585, defined as injection, onabotulinumtoxinA, 1 unit. The administration procedure is billed separately with the CPT code tied to the indication and anatomy. CMS coding guidance for botulinum toxins says providers should submit the relevant administration code with the drug HCPCS code and align diagnosis coding accordingly. CMS article on botulinum toxins (<https://www.cms.gov/medicare-coverage-database/view/article.aspx?articleId=52498>)

Highest-yield CPT codes to know

CPT	Common AbbVie-relevant use case	Coding note
64615	Chronic migraine	High-yield migraine code paired with J0585 in CMS coding guidance.
52287	Overactive bladder, urge incontinence, neurogenic bladder	Important for urology and bladder use cases with J0585.
64612	Blepharospasm, hemifacial spasm	Useful for facial muscle administration.
64616	Cervical dystonia	High-yield neck-muscle chemodenervation code.
64642-64647	Limb spasticity	Important because code selection varies by number and location of muscles treated.
67345	Strabismus chemodenervation	Less central, but still interview-relevant.
64617	Laryngeal dystonia / spasmodic dysphonia context	Mention carefully as indication-specific procedural coding.

Source: CMS article on botulinum toxins (<https://www.cms.gov/medicare-coverage-database/view/article.aspx?articleId=52498>)

Modifier rules that matter

- Use JW when part of a separately payable Part B drug from a single-dose container is discarded.
- Use JZ when the drug came from a single-dose container and no amount was discarded.
- Since October 1, 2023, claims for separately payable drugs from single-dose containers that omit the required JW or JZ reporting may be returned as unprocessable.
- When there is waste, Medicare expects the administered amount on one line and the discarded amount on a second line with JW.
- When there is no waste, the claim uses a single line with JZ.
- Overfill is never billable and must not be reported with JW.

Source: CMS JW/JZ FAQ (<https://www.cms.gov/files/document/jw-modifier-jz-modifier-policy-faqs.pdf>)

Bilateral and anatomic nuance

- CPT 64611 and 64615 have MPFS bilateral status indicator 2, so modifier 50 should not be used.
- CPT 64612, 64616, 64617, and 67345 have bilateral status indicator 1, so modifier 50 may apply when performed bilaterally.
- CPT 52287 and 64642-64647 have bilateral status indicator 0, so modifier 50 should not be reported.
- CMS coding guidance also references anatomic modifiers such as LT, RT, or modifier 59 where appropriate.

Source: CMS article on botulinum toxins (<https://www.cms.gov/medicare-coverage-database/view/article.aspx?articleId=52498>), CMS MPFS Relative Value Files landing page (<https://www.cms.gov/medicare/payment/fee-schedules/physician-fee-schedule/search>)

Why waste is a real AbbVie issue

The FDA label says BOTOX is supplied in single-dose vials and that any unused portion must be discarded. That creates routine leftover-volume risk whenever the ordered dose does not match vial configuration exactly. For providers, the practical issues are claim correctness, auditability, and recoverable payment for discarded labeled units. For AbbVie, the issue goes one step further because discarded units also feed the federal manufacturer refund program for discarded drug. FDA BOTOX label (https://www.accessdata.fda.gov/drugsatfda_docs/label/2023/103000s5327lbl.pdf), CMS JW/JZ FAQ (<https://www.cms.gov/files/document/jw-modifier-jz-modifier-policy-faqs.pdf>)

What you can say about AbbVie's refund exposure

You cannot calculate AbbVie's actual Botox refund obligation from public sources alone. What you can say precisely is:

- The refund is not assessed per provider claim.
- It is a quarterly manufacturer refund.
- It is triggered only to the extent discarded units exceed the applicable threshold.
- CMS states the applicable percentage must be at least 10 percent.
- The calculation uses discarded units and the quarter's payment amount.

The interview-safe formula is:

Refund owed for the quarter = max(0, (discarded J0585 units x payment amount) - (applicable percentage x estimated total allowed charges))

Source: CY 2026 PFS fact sheet

(<https://www.cms.gov/newsroom/fact-sheets/calendar-year-cy-2026-medicare-physician-fee-schedule-final-rule-cms-1832-f>), CMS discarded-drug refund program rulemaking summary (<https://www.cms.gov/medicare/payment/fee-schedules/physician-fee-schedule/part-b-discarded-drug-refunds>)

Short interview-ready Botox answer

"For therapeutic Botox, the drug line is J0585 billed per unit, and the administration is coded separately with the indication-specific CPT, such as 64615 for chronic migraine, 52287 for bladder, 64612 for blepharospasm or hemifacial spasm, 64616 for cervical dystonia, and 64642 through 64647 for limb spasticity. Because Botox is single-dose only, the waste modifiers matter. If there is discarded drug, the administered amount goes on one line and the discarded amount goes on a second line with JW. If there is no discarded amount, the claim uses JZ. Overfill is not billable. For AbbVie, that matters not just for clean provider billing but because discarded-unit reporting also connects to the federal discarded-drug manufacturer refund program." CMS article on botulinum toxins (<https://www.cms.gov/medicare-coverage-database/view/article.aspx?articleId=52498>), CMS JW/JZ FAQ (<https://www.cms.gov/files/document/jw-modifier-jz-modifier-policy-faqs.pdf>)

6. Where Part A, IPPS, and Hospital Reimbursement Actually Matter

The key nuance

Botox itself is usually a Part B office or hospital-outpatient reimbursement problem, not a classic inpatient IPPS problem. CMS's national JW/JZ FAQ says the modifiers are mostly reported in physician offices, hospital outpatient departments, and CAHs when the drug is separately payable. That is the cleanest way to explain why Botox is central to the role but not necessarily evidence that AbbVie has a flagship IPPS-heavy Botox reimbursement issue. CMS JW/JZ FAQ (<https://www.cms.gov/files/document/jw-modifier-jz-modifier-policy-faqs.pdf>)

The strongest AbbVie examples for Part A and hospital competency

Product	Why hospital / Part A knowledge matters
Duopa	PEG-J placement, inpatient vs outpatient initiation, and DME/home-infusion transition make it the strongest portfolio example of hospital-procedural reimbursement complexity.
Skyrizi	IV induction creates hospital outpatient and infusion-center economics before the patient transitions to maintenance therapy.
Elahere	Infused oncology products can create hospital and facility reimbursement implications even when the dominant payment route is outpatient Part B.
Ozurdex	Procedural administration and facility economics matter more than retail or traditional Part D logic.

Duopa is the cleanest interview example

The FDA label says Duopa is delivered through a PEG-J tube for long-term administration and can be initiated temporarily through a naso-jejunal tube. That means the reimbursement problem is not just the drug. It includes procedural setting, site of care, possible inpatient vs outpatient initiation, and transition into pump- and DME-related support. Noridian materials also show the product's interaction with external infusion pump coverage logic, which is why Duopa is a much cleaner Part A / hospital-adjacent example than Botox itself. DUOPA label (https://www.accessdata.fda.gov/drugsatfda_docs/label/2024/203951s027lbl.pdf), Noridian external infusion pump LCD page (<https://med.noridianmedicare.com/web/jddme/policies/lcd/active/I33794>)

Interview-ready Part A answer

"For AbbVie, Part A is probably less about having a massive inpatient-only drug franchise and more about needing someone who understands hospital reimbursement when administration or initiation shifts into the inpatient or facility setting. Botox itself is mostly a Part B office and hospital outpatient coding problem. The stronger Part A and hospital-adjacent examples in the portfolio are products like Duopa, where PEG-J placement and initiation create procedural and setting-of-care complexity, plus products like Skyrizi IV induction, Elahere, and Ozurdex, where hospital and facility reimbursement knowledge still matters." DUOPA label (https://www.accessdata.fda.gov/drugsatfda_docs/label/2024/203951s027lbl.pdf), AbbVie 2025 Form 10-K (<https://www.sec.gov/Archives/edgar/data/1551152/000155115226000008/abbv-20251231.htm>)

7. Joseph Stewart Fit and Behavioral Proof Points

Best fit framing

Joseph's strongest fit is that he already works like a reimbursement strategist, not just a policy analyst. The Biocon-oriented executive summary you provided is useful here as a structure because it highlights the right operating themes: launch readiness, dual-channel strategy, provider and payer pathway design, reimbursement support operations, analytics, and policy-to-execution translation. Those are the right interview themes for AbbVie too, even though the product facts are different. Executive Summary.pdf (/Users/josephstewart/Documents/Executive%20Summary.pdf)

AbbVie-relevant proof points

Proof point	Why it matters at AbbVie	How to phrase it
Consulting work across manufacturers, PBMs, payers, biosimilars, and generics	Shows broad channel and reimbursement perspective.	"I am used to translating policy and reimbursement mechanics into commercially usable strategy."
Otsuka reimbursement and health policy leadership	Shows internal manufacturer experience and cross-functional execution.	"I have worked inside the manufacturer environment and know how to align policy, access, and business stakeholders."
Part D, PBM, and formulary analytics depth	Distinguishes Joseph for AbbVie's Part D-heavy brands.	"I bring unusually strong plan-side and formulary-side literacy for a reimbursement strategy role."
Provider-economics and reimbursement architecture work	Connects directly to Part B and buy-and-bill logic.	"I focus on whether reimbursement architecture supports adoption, not just whether coverage exists on paper."
Claims and adjudication exposure	Makes the operational coding and workflow discussion more credible.	"I have seen how reimbursement breaks in practice, not just how it is supposed to work."

Where to stay disciplined

- Do not claim direct CMS negotiation execution unless specifically supported.
- Do not claim direct AbbVie product ownership.
- Do not overclaim deep prior buy-and-bill line ownership if the background is more adjacent than direct.
- Do show confidence on Medicare strategy, reimbursement mechanics, payer behavior, and launch readiness.

8. Questions You Should Be Ready For

Why are you a fit for this role?

Anchor on policy-to-access translation, Medicare fluency, and product-level reimbursement strategy.

How would you approach AbbVie's Medicare Part B reimbursement strategy?

Anchor on a product heat map, Botox 2028 readiness, ASP and payment-limit monitoring, discarded-drug policy, and portfolio-wide CMS tracking.

How do you think about Part B and Part D together for AbbVie?

Anchor on Skyrizi IV to SC transition, Venclexta regimen economics, and negotiation spillover from Part D to Part B planning.

What does AbbVie need to be thinking about as negotiation expands into Part B?

Anchor on the difference between price policy and operational reimbursement: ASP, payment-limit files, provider spread, claims workflow, and field messaging.

What would your first 90 days look like?

Anchor on a Medicare heat map, Botox workstream, reimbursement playbook refresh, and one integrated CMS tracker across negotiation, ASP, coding, and discarded-drug issues.

9. Rapid-Fire Talking Points

- Botox is the most obvious current AbbVie Part B reimbursement flashpoint.
- J0585 is billed per unit, and administration is coded separately by indication-specific CPT.
- Botox waste matters because it is single-dose only, and JW/JZ accuracy affects both provider payment and manufacturer refund exposure.
- Botox is mostly a Part B office and HOPD issue, not a classic IPPS coding issue.
- Duopa is the stronger hospital and Part A-adjacent example in AbbVie's portfolio.
- Skyrizi is the cleanest cross-benefit example because it crosses from IV induction to subcutaneous maintenance.
- AbbVie is already living through three phases of negotiation exposure: Imbruvica 2026, Vraylar and Linzess 2027, Botox 2028.
- The role is valuable because it connects federal policy, reimbursement operations, and commercial strategy in one seat.

10. Sources

Primary public sources

- AbbVie 2025 Form 10-K (<https://www.sec.gov/Archives/edgar/data/1551152/000155115226000008/abbv-20251231.htm>)
- CMS IPAY 2026 negotiated prices (<https://www.cms.gov/files/document/fact-sheet-negotiated-prices-initial-price-applicability-year-2026.pdf>)
- CMS IPAY 2027 negotiated prices (<https://www.cms.gov/files/document/fact-sheet-negotiated-prices-ipay-2027.pdf>)
- CMS IPAY 2028 selected-drug list (<https://www.cms.gov/files/document/factsheet-medicare-negotiation-selected-drug-list-ipay-2028.pdf>)
- CMS IPAY 2028 final guidance (<https://www.cms.gov/files/document/ipay-2028-final-guidance.pdf>)
- CMS April 2026 Part B payment limits (<https://www.cms.gov/files/zip/april-2026-medicare-part-b-payment-limit-files-03-30-2026-final-file.zip>)
- CMS Part B drugs overview (<https://www.cms.gov/cms-guide-medical-technology-companies-and-other-interested-parties/payment/part-b-drugs>)
- CMS article on botulinum toxins (<https://www.cms.gov/medicare-coverage-database/view/article.aspx?articleId=52498>)
- CMS JW/JZ FAQ (<https://www.cms.gov/files/document/jw-modifier-jz-modifier-policy-faqs.pdf>)
- CMS discarded-drug refund landing page (<https://www.cms.gov/medicare/payment/fee-schedules/physician-fee-schedule/part-b-discarded-drug-refunds>)
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- FDA BOTOX label (https://www.accessdata.fda.gov/drugsatfda_docs/label/2023/103000s5327lbl.pdf)
- FDA SKYRIZI label (https://www.accessdata.fda.gov/drugsatfda_docs/label/2024/761105s032lbl.pdf)
- FDA ELAHERE label (https://www.accessdata.fda.gov/drugsatfda_docs/label/2022/761310s000lbl.pdf)
- FDA DUOPA label (https://www.accessdata.fda.gov/drugsatfda_docs/label/2024/203951s027lbl.pdf)
- Noridian external infusion pump LCD page (<https://med.noridianmedicare.com/web/jddme/policies/lcd/active/133794>)

Internal source materials used for synthesis

This packet also synthesizes the user's prior AbbVie working materials, including Executive Summary.pdf (/Users/josephstewart/Documents/Executive%20Summary.pdf), abbvie (../abbvie), abbvie_director_government_affairs_interview_brief.md (./abbvie_director_government_affairs_interview_brief.md), AbbVie_Director_Government_Affairs_US_Reimbursement_Interview_Prep.pdf (./pdf/AbbVie_Director_Government_Affairs_US_Reimbursement_Interview_Prep.pdf), app/about/page.tsx (../app/about/page.tsx), and portfolio_brief.md (../portfolio_brief.md).